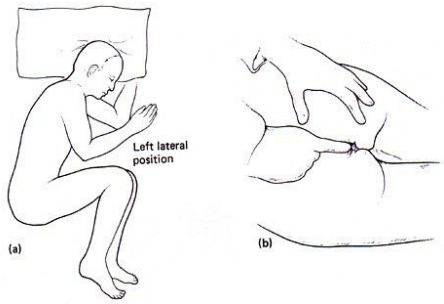
Digital Rectal Examination (DRE) — OSCE Checklist

Deakin MD2 · RG5 · Part-task trainer

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Consent → Position → Inspect → Strain → Insert → Sweep walls → Anterior wall → Tone → Inspect glove → Close**. *Treat the model as a real patient: communicate, reassure and explain what they will feel.*

1. Before you start

☐	Wash hands; fingernails short; introduce self; confirm patient ID
☐	Informed consent **before** patient undresses: why, what you will do, position, what they will feel
☐	Do not assume willingness because they have not objected; allow time for questions
☐	Offer a chaperone; privacy (curtains); quiet, well-lit, warm room
☐	Check cautions — seek senior advice if: anal fissure, thrombosed prolapsed haemorrhoids, child, active IBD / diverticulitis, recent surgery/trauma/radiotherapy, immunocompromised or bleeding risk, history of abuse
☐	Equipment: **2 pairs** of non-sterile gloves, lubricant, light, bluey, tissues
☐	Position: **left lateral**, knees drawn up to chest; cover with gown/sheet until exposure needed



Left lateral position for DRE

2. Procedure

	Step	What to do / say	Abnormal
Inspect (gloved, separate buttocks)			
☐	Perianal area	Skin tags, erythema, excoriation, warts (condylomata), fissure, external/thrombosed haemorrhoids, prolapse, carcinoma	
☐	Strain	"Push down as if opening your bowels"	Incontinence, perineal descent, haemorrhoids, prolapse
Insert			
☐	Warn the patient	"You may feel like you need to open your bowels — it's just the feeling of my finger"	
☐	Lubricate	Lubricate right index finger; lift right buttock with left hand	
☐	Position finger	Pad of finger on anus, pointing towards patient's front	
☐	Relax and enter	Patient breathes deeply through mouth; wait for sphincter to relax, then gently insert, turning finger towards head	Internal sphincter tone assessed on entry
☐	Resistance	Stop, hold still, let patient breathe slowly; do **not** force. Stop if spasm and severe pain	Fissure, stricture, fibrosis, carcinoma
Palpate			
☐	Rectal walls	Sweep pulp from 2 o'clock clockwise to 10 o'clock at 3 levels (~7–8 cm); feel sacral hollow and coccyx	Mass, polyp, ulcer with hard raised edge
☐	Anterior wall — male	10 to 2 o'clock: prostate size, consistency, median sulcus, nodules, tenderness ("you may feel you need to pass urine")	See box
☐	Anterior wall — female	Cervix felt as a smooth lump; retroverted uterus may press on wall	Pelvic mass
☐	Strain again	Patient pushes down; sweep to feel an extra centimetre	
☐	Contents	Consistency of faeces	Hard impacted stool, foreign body
☐	Squeeze	"Relax… now squeeze my finger… now relax"	External sphincter tone; weak = sphincter injury or neurological
After removal			
☐	Inspect glove	Colour, blood, mucus, pus	Fresh blood = haemorrhoids, rectal lesion; black = upper GI bleed, iron
☐	FOBT*	Faecal occult blood test if indicated	

3. Closing

☐	Offer tissues / help wiping; remove outer gloves when soiled
☐	Dispose of waste in contaminated bin
☐	Cover patient; tell them it's finished; offer help dressing
☐	Simple explanation of findings; if unsure, say you'll confirm with the treating doctor
☐	Invite questions; check understanding; wash hands
☐	Document consent, chaperone and all findings

4. Quick reference

Finding	Think of
Loss of tone, patulous anus	Sphincter injury, cord compression / cauda equina
Hard impacted stool	Constipation, overflow incontinence
Irregular mucosa / mass	Rectal cancer
Smooth, large, firm, non-tender prostate	BPH
Hard irregular nodule / fixed mass	Prostate cancer, chronic prostatitis
Large, boggy, tender prostate	Acute prostatitis
Mass on anterior wall (female)	Ovarian or uterine mass

#	Indications
1	Rectal bleeding
2	Anal symptoms: itch, pain, burning
3	Change in bowel habit
4	Urinary symptoms (prostate)
5	Incontinence
6	Suspected spinal cord compression — check anal tone

• Normal prostate: rubbery, firm, bilobed, smooth with a central median sulcus. With age it enlarges and the sulcus is lost; cancer is hard and irregular.
• Internal sphincter = smooth muscle (involuntary, on entry). External sphincter = skeletal muscle (voluntary, "squeeze").